Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1 to 6**, choose the answer (**A**, **B** or **C**) which you think fits best according to the text.

Write your answers on the separate **Answer Sheet**.

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1. The purpose of the memo about IV solution bags is to remind health practitioners

A of the procedures to follow when using them.
B of the hazards associated with faulty ones.
C why they shouldn't be reused.

Memo to staff - Intravenous solution bags
IV fluids are administered via a plastic IV solution bag which collapses on itself as it empties. When a bag is disconnected by removing the giving set spike, air can enter the bag. If it is then reconnected to an IV line, air can potentially enter the patient's vein and cause an air embolism. For this reason, partially used IV bags must never be re-spiked. All IV bags are designed for single use only - for use in one patient and on one occasion only. All registered large volume injections, including IV bags, are required to have this warning (or words to the same effect) clearly displayed on the labelling. In addition to the potential risk of introducing an air embolus, re-spiking can also result in contamination of the fluid, which may lead to infection and bacteraemia.

2. What do we learn about the use of TENS machines?

A Evidence for their efficacy is unconfirmed.

B They are recommended in certain circumstances.

C More research is needed on their possible side effects.

Update on TENS machines
The Association of Chartered Physiotherapists in Women's Health has an expert panel which could not find any reports suggesting that negative effects are produced when TENS has been used during pregnancy. However, in clinical practice, TENS is not the first treatment of choice for women presenting with musculoskeletal pain during pregnancy. The initial treatment should be aimed at correcting any joint or muscle dysfunction, and a rehabilitation programme should be devised. However, if pain remains a significant factor, then TENS is preferable to the use of strong medication that could cross the placental barrier and affect the foetus. No negative effects have been reported following the use of this modality during any of the stages of pregnancy. Therefore, TENS is preferable for the relief of pain.

3. If surgical instruments have been used on a patient suspected of having prion disease, they

A must be routinely destroyed as they cannot be reused.

B may be used on other patients provided the condition has been ruled out.

C should be decontaminated in a particular way before use with other patients.

Guidelines: Invasive clinical procedures in patients with suspected prion disease
It is essential that patients suspected of suffering from prion disease are identified prior to any surgical procedure. Failure to do so may result in exposure of individuals on whom any surgical equipment is subsequently used. Prions are inherently resistant to commonly used disinfectants and methods of sterilisation. This means that there is a possibility of transmission of prion disease to other patients, even after apparently effective methods of decontamination or sterilisation have been used. For this reason, it may be necessary to destroy instruments after use on such a patient, or to quarantine the instrument until the diagnosis is either confirmed, or an alternative diagnosis is established. In any case, the instruments can be used for the same patient on another occasion if necessary.

4. The email suggests that POCT devices

A should only be used in certain locations.

B must be checked regularly by trained staff.

C can produce results that may be misinterpreted.

To: All Staff

Subject: Management of Point of Care Testing (POCT) Devices

Due to several recent incidents associated with POCT devices, staff are requested to read the following advice from the manufacturer of the devices.

The risks associated with the use of POCT devices arise from Management of Point of Care Testing Devices Version 4 January 2014, the inherent characteristics of the devices themselves and from the interpretation of the results they provide. They can be prone to user errors arising from unfamiliarity with equipment more usually found in the laboratory. User training and competence is therefore crucial.

5. It's permissible to locate a baby's identification band somewhere other than the ankles when

A the baby is being moved due to an emergency.

B the bands may interfere with treatment.

C the baby is in an incubator.

Identification bands for babies

The identification bands should be located on the baby's ankles with correct identification details unless the baby is extremely premature and/or immediate vascular access is required. If for any reason the bands need to be removed, they should be relocated to the wrists or if this is not possible, fixed visibly to the inside of the incubator. Any ill-fitting or missing labels should be replaced at first check. Identity bands must be applied to the baby's ankles at the earliest opportunity as condition allows and definitely in the event of fire evacuation or transportation.

6. What is the memo doing?

A providing an update on the success of new guidelines

B reminding staff of the need to follow new guidelines

C announcing the introduction of new guidelines

Memo: Administration of antibiotics
After a thorough analysis and review, our peri-operative services, in conjunction with the Departments of Surgery and Anaesthesia, decided to change the protocols for the administration of pre-operative antibiotics and established a series of best practice guidelines. This has resulted in a significant improvement in the number of patients receiving antibiotics within the recommended 60 minutes of their incision. A preliminary review of the total hip and knee replacements performed in May indicates that 88.9% of patients received their antibiotics on time.